

Health Insurance Coverage in the United States: 2025

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By Lisa N. Bunch and Halelujha Ketema

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Health Insurance Coverage in the United States: 2025

INTRODUCTION

The U.S. Census Bureau collects data and publishes estimates on health insurance coverage to evaluate national trends in access to health care. Health insurance reduces exposure to financial risks from unexpected medical expenses, offers a means for financing an individual's health care expenses, and provides more economic stability for people and families. From year to year, the prevalence of health insurance coverage and the distribution of coverage types may change due to economic trends, shifts in the demographic composition of the population (such as population aging), and policy changes that affect access to care.

The estimates in this report are based on data collected in the 2026 and earlier Current Population Survey Annual Social and Economic Supplements (CPS ASEC) conducted by the Census Bureau. This report presents statistics on health insurance coverage in the United States in 2025 and changes in health insurance coverage rates between 2024 and 2025.* Estimates for 2024 in this report may not match those published last year due to

* The U.S. Census Bureau has reviewed this data product to ensure appropriate access, use, and disclosure avoidance protection of the confidential source data used to produce this product (Data Management System [DMS] number: P-7534374 and Disclosure Review Board [DRB] approval number: CBDRB-FY26-0281). To further protect respondent privacy, all estimates in this report have undergone additional rounding. As a result, details may not sum to totals. All comparative statements have undergone statistical testing and are statistically significant at the 90 percent confidence level unless otherwise noted.

the implementation of Vintage 2025 population controls.¹

This report is released alongside two other reports focused on household income and poverty in the United States. These estimates can be found in "Income in the United States: 2025" and "Poverty in the United States: 2025."²

HIGHLIGHTS

- In 2025, 26.7 million people (7.9 percent) were uninsured for the entire year, not statistically different from 2024 (Table 1 and Figure 1).^{3,4}
- Of the subtypes of health insurance coverage, employment-based insurance was the most common, covering 53.5 percent of the population for some or all of the calendar year, followed by Medicare (20.1 percent), Medicaid (17.1 percent), direct-purchase coverage (10.5 percent), TRICARE (2.8 percent), and VA and CHAMPVA coverage (1.2 percent).⁵
- The share of individuals covered by public health insurance was 35.8 percent in 2025, not statistically different from 2024. While Medicare coverage increased by 0.6 percentage points, Medicaid coverage decreased by 0.5 percentage points.
- Public coverage for adults ages 19 to 64 declined by 0.5 percentage points in 2025 to 17.3 percent. This was driven by a 0.6 percentage-point decrease in Medicaid coverage for adults ages 19 to 64 (Figure 2).⁶

- In 2025, 41.5 percent of adults age 65 and older held private coverage, down 2.8 percentage points from 2024. This decrease was driven by a 1.5 percentage-point drop in employment-based coverage and a 1.4 percentage-point drop in direct-purchase coverage for adults age 65 and older.⁷

ESTIMATES OF HEALTH INSURANCE COVERAGE IN THE UNITED STATES

This report classifies health insurance coverage into three different categories: overall coverage, private coverage, and public coverage (refer to the "What Is Health Insurance Coverage?" text box). In the CPS ASEC, people are considered insured if they were covered by any type of health insurance for some or all of the previous calendar year. People are considered uninsured if they were not covered by any type of insurance for the entire year.^{8,9}

In 2025, 26.7 million people (7.9 percent) were uninsured for the entire year (Table 1 and Figure 1). That means most people (92.1 percent) had health insurance coverage at some point during the calendar year. More people had private health insurance (65.7 percent) than public coverage (35.8 percent).

There were no statistically significant changes in private coverage, public coverage, or the uninsured rate between 2024 and 2025.

Employment-based insurance was the most common subtype of health insurance in the civilian noninstitutionalized population (53.5 percent), followed by Medicare (20.1 percent), Medicaid (17.1 percent), direct-purchase insurance (10.5 percent), TRICARE (2.8 percent), and VA and CHAMPVA health care (1.2 percent).¹⁰

There were no significant changes between 2024 and 2025 in any of the subtypes of private health insurance (employment-based coverage, direct-purchase coverage, and TRICARE coverage).

Of the subtypes of public health insurance, Medicare coverage increased by 0.6 percentage points to 20.1 percent in 2025. The increase in the Medicare rate was due, in part, to growth in the number of people age 65 and older.¹¹ The increase in Medicare coverage was offset by a decline in Medicaid coverage by 0.5 percentage points, which resulted in no change in the public coverage rate between 2024 and 2025.

HEALTH INSURANCE COVERAGE BY TYPE AND SELECTED CHARACTERISTICS

Health Insurance Coverage by Age

Age is associated with the likelihood that a person has health insurance coverage, as well as with health coverage type. In general, older adults (age 65 and older) and children (under the age of 19) are more likely to have health insurance coverage than those ages 19 to 64, in part because their age makes them eligible for certain public health insurance programs. Medicare

What Is Health Insurance Coverage?

Health insurance coverage in the Current Population Survey Annual Social and Economic Supplement (CPS ASEC) refers to comprehensive coverage at any time during the calendar year for the civilian noninstitutionalized population of the United States.¹ For reporting purposes, the U.S. Census Bureau broadly classifies health insurance coverage as private insurance or public insurance.

Private Coverage

- **Employment-based:** Plan provided through an employer or union.
- **Direct-purchase:** Coverage purchased directly from an insurance company, or through a federal or state Marketplace (e.g., healthcare.gov).
- **TRICARE:** Coverage through TRICARE, formerly known as Civilian Health and Medical Program of the Uniformed Services.

Public Coverage

- **Medicare:** Federal program that helps to pay health care costs for people age 65 and older and for certain people under the age of 65 with long-term disabilities.
- **Medicaid:** This report uses the term “Medicaid” to include the specific Medicaid government program and other programs for low-income individuals administered by the states, such as the Children’s Health Insurance Program (CHIP) and Basic Health Programs.
- **VA and CHAMPVA:** Care provided by the Department of Veterans Affairs, the military, and the Civilian Health and Medical Program of the Department of Veterans Affairs.

Additionally, people are considered uninsured if they only had coverage through the Indian Health Service (IHS), as IHS coverage is not considered comprehensive.

¹ Comprehensive health insurance covers basic health care needs. This definition excludes single-service plans such as accident, dental, disability, prescription medicine, or vision plans.

provides health coverage benefits for most adults age 65 and older. Children under the age of 19 may qualify for coverage through Medicaid or the Children’s Health Insurance Program (CHIP), or they may be covered as dependents on a parent or guardian’s health

insurance plan.¹² Additionally, the Patient Protection and Affordable Care Act (ACA) extended coverage through a parent or guardian’s plan to children and young adults until the age of 26.¹³

Table 1.

Number of People by Health Insurance Coverage Status and Type: 2024 and 2025

(Numbers in thousands. Margins of error in thousands. Population as of March of the following year. Information on confidentiality protection, sampling error, nonsampling error, and definitions is available at <<https://www2.census.gov/programs-surveys/cps/techdocs/cpsmar26.pdf>>)

Coverage type	2024 ¹		2025		Year-to-year change (2025 less 2024)
	Number	Margin of error ² (±)	Number	Margin of error ² (±)	
Total	336,800	153	337,900	139	*1,108
Any health plan³	309,900	635	311,200	779	*1,376
Any private plan	222,100	1,362	222,000	1,453	-97
Employment-based	180,500	1,295	180,700	1,432	219
Direct-purchase	36,260	767	35,530	817	-737
Marketplace coverage ...	14,480	535	14,260	553	-230
TRICARE	9,585	510	9,294	521	-291
Any public plan	120,800	1,067	121,100	1,147	340
Medicare	65,740	435	67,910	478	*2,165
Medicaid	59,220	1,042	57,800	1,063	*-1,418
VA and CHAMPVA ⁴	3,873	221	4,212	253	339
Uninsured⁵	26,940	622	26,670	764	-268

* An asterisk preceding an estimate indicates change is statistically different from zero at the 90 percent confidence level.

¹ Estimates for 2024 may not match those published last year due to the implementation of Vintage 2025 population controls.

² A margin of error (MOE) is a measure of an estimate's variability. The larger the MOE in relation to the size of the estimate, the less reliable the estimate. This number, when added to and subtracted from the estimate, forms the 90 percent confidence interval. MOEs shown in this table are based on standard errors calculated using replicate weights.

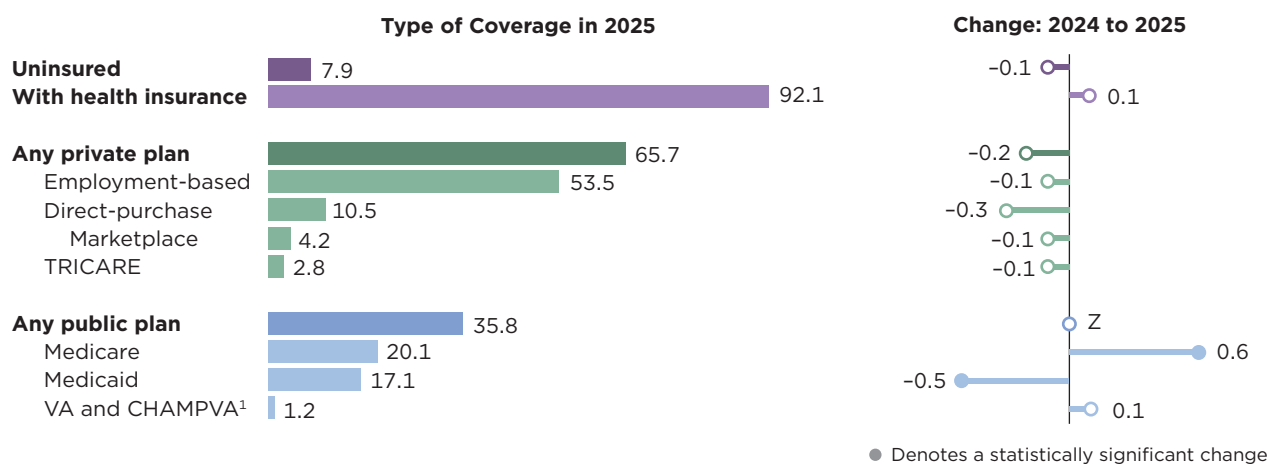
³ The estimates by type of coverage are not mutually exclusive; people can be covered by more than one type of health insurance during the year.

⁴ Includes CHAMPVA (Civilian Health and Medical Program of the Department of Veterans Affairs), as well as care provided by the Department of Veterans Affairs (VA) and the military.

⁵ In the CPS ASEC, individuals are considered to be uninsured if they did not have health insurance coverage for the entire calendar year.

Source: U.S. Census Bureau, Current Population Survey, 2025 and 2026 Annual Social and Economic Supplements (CPS ASEC; Data Management System [DMS] number: P-7534374 and Disclosure Review Board [DRB] approval number: CBDRB-FY26-0281).

Figure 1.

Percentage of People by Type of Health Insurance Coverage: 2024 and 2025

Z Rounds to zero.

¹ Includes CHAMPVA (Civilian Health and Medical Program of the Department of Veterans Affairs), as well as care provided by the Department of Veterans Affairs (VA) and the military.

Note: Population as of March of the following year. The estimates by type of coverage are not mutually exclusive; people can be covered by more than one type of health insurance during the year. Margins of error and other related estimates are available in Table 2 at <www.census.gov/library/publications/2026/demo/p60-291.html>. Information on confidentiality protection, sampling error, nonsampling error, and definitions is available at <<https://www2.census.gov/programs-surveys/cps/techdocs/cpsmar26.pdf>>.

Source: U.S. Census Bureau, Current Population Survey, 2025 and 2026 Annual Social and Economic Supplements (CPS ASEC). (Data Management System [DMS] number: P-7534374 and Disclosure Review Board [DRB] approval number: CBDRB-FY26-0281).

Private Coverage by Age

Figure 2 shows health insurance coverage rates by major age category. While there were no significant changes to private coverage rates for children under the age of 19 and adults ages 19 to 64, private coverage declined 2.8 percentage points for adults age 65 and older between 2024 and 2025. This decrease was driven by a 1.5 percentage-point drop in employment-based coverage and a 1.4 percentage-point drop in direct-purchase coverage for adults age 65 and older.¹⁴

Public Coverage by Age

In 2025, about one-third of children under the age of 19 had public health coverage (33.7 percent), compared with 17.3 percent of adults ages 19 to 64. Most adults age 65 and older (94.3 percent) held public coverage.

Between 2024 and 2025, the public coverage rate decreased by 0.5 percentage points for

adults ages 19 to 64, driven by a 0.6 percentage-point decrease in Medicaid coverage.

For adults age 65 and older, 7.4 percent held Medicaid coverage at some point in 2025, an increase of 0.7 percentage points from 2024.

Private and Public Health Insurance Coverage by Selected Characteristics

Figure 3 examines private and public coverage by selected characteristics. Overall, neither private nor public health insurance rates changed significantly between 2024 and 2025. Private health insurance coverage declined for non-Hispanic White individuals and native-born individuals between 2024 and 2025.¹⁵ For people of all ages, there were no significant changes for the other groups displayed in Figure 3.¹⁶

Among adults ages 19 to 64, the public coverage rate decreased by 0.5 percentage points between 2024 and 2025, with significant

decreases in public coverage for all workers and for adults with no disability.¹⁷

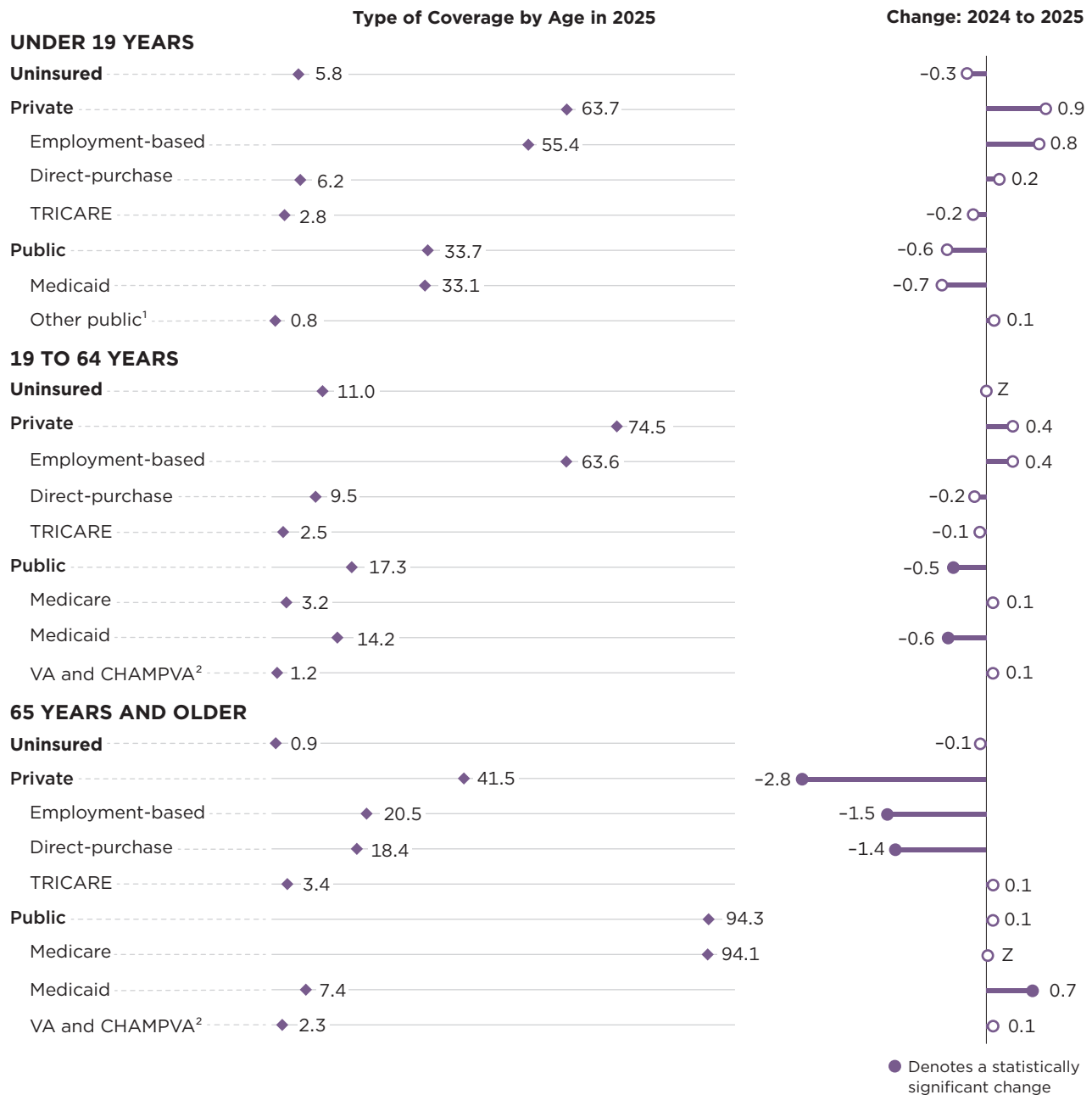
Uninsured Rates for Children and Working-Age Adults by Selected Characteristics

For children under the age of 19 and working-age adults ages 19 to 64, health insurance coverage status and coverage type vary. While people age 65 and older have nearly universal health insurance coverage (due to access to Medicare), those under the age of 65 are more likely to have variability in their type and continuity of coverage over time.

Figure 4 presents the uninsured rates for 2024 and 2025 by age group. Among working-age adults ages 19 to 64, those ages 19 to 25 had the highest uninsured rate (14.5 percent) in 2025, and those ages 45 to 64 had the lowest (8.6 percent). Overall, the uninsured rate for adults decreased as age increased.

Figure 2.

Percentage of People With Selected Coverage Types and Uninsured by Age Group: 2024 and 2025



Z Rounds to zero.

¹ Other public includes Medicare, Civilian Health and Medical Program of the Department of Veterans Affairs (CHAMPVA), and care provided by the Department of Veterans Affairs (VA) and the military.

² Includes CHAMPVA, as well as care provided by the VA and the military.

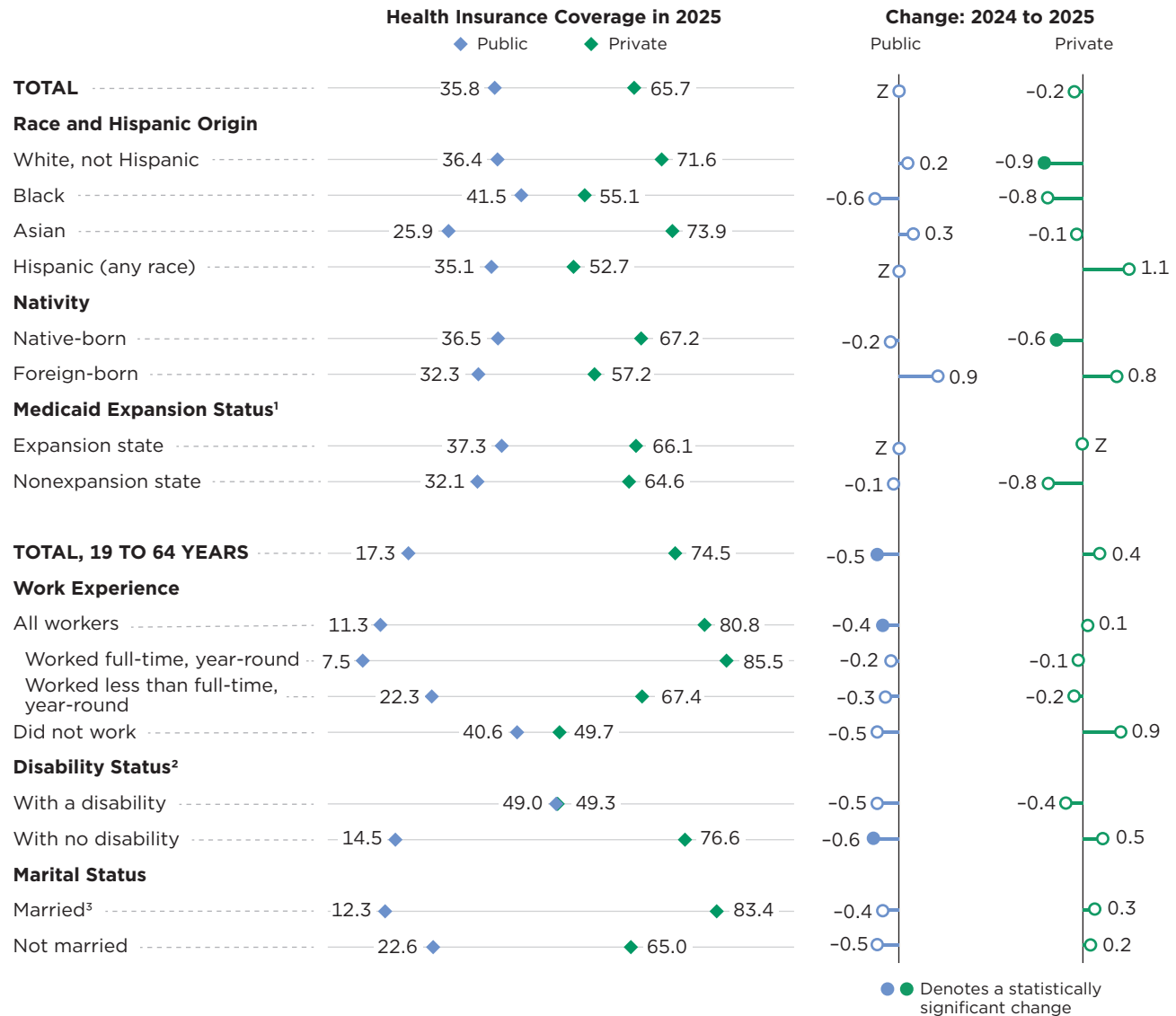
Note: Population as of March of the following year. The estimates by type of coverage are not mutually exclusive; people can be covered by more than one type of health insurance during the year. Margins of error and other related estimates are available in Table 2 at www.census.gov/library/publications/2026/demo/p60-291.html. Information on confidentiality protection, sampling error, nonsampling error, and definitions is available at <https://www2.census.gov/programs-surveys/cps/techdocs/cpsmar26.pdf>.

Source: U.S. Census Bureau, Current Population Survey, 2025 and 2026 Annual Social and Economic Supplements (CPS ASEC). (Data Management System [DMS] number: P-7534374 and Disclosure Review Board [DRB] approval number: CBDRB-FY26-0281).

Figure 3.

Health Insurance Coverage Type by Selected Characteristics: 2024 and 2025

(In percent)



Z Rounds to zero.

¹ Indicates residence in one of the 40 expansion states and the District of Columbia, or 10 nonexpansion states. The states that had not expanded Medicaid as of January 1, 2025, are AL, FL, GA, KS, MS, SC, TN, TX, WI, and WY. For more information, refer to <www.medicaid.gov/state-overviews>.

² The sum of those with and without a disability does not equal the total because disability status is not defined for individuals in the U.S. armed forces.

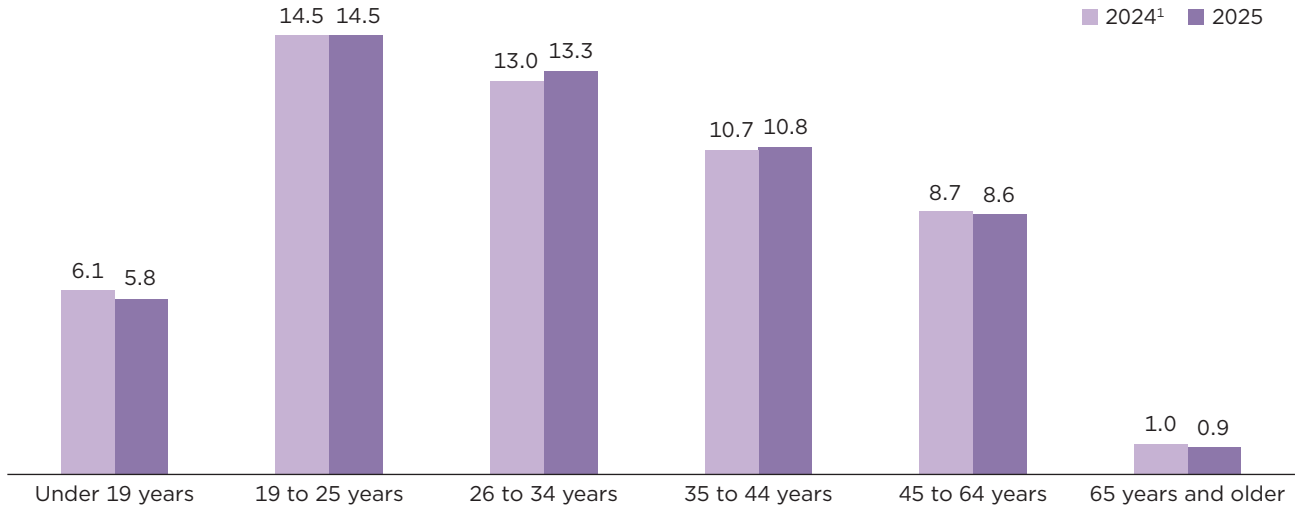
³ The combined category "married" includes three individual categories: "married, civilian spouse present," "married, U.S. armed forces spouse present," and "married, spouse absent."

Note: Population as of March of the following year. The estimates by type of coverage are not mutually exclusive; people can be covered by more than one type of health insurance during the year. Margins of error and other related estimates are available in Tables 3 and 4 at <www.census.gov/library/publications/2026/demo/p60-291.html>. Information on confidentiality protection, sampling error, nonsampling error, and definitions is available at <<https://www2.census.gov/programs-surveys/cps/techdocs/cpsmar26.pdf>>.

Source: U.S. Census Bureau, Current Population Survey, 2025 and 2026 Annual Social and Economic Supplements (CPS ASEC). (Data Management System [DMS] number: P-7534374 and Disclosure Review Board [DRB] approval number: CBDRB-FY26-0281).

Figure 4.

Percentage of People Without Health Insurance Coverage by Age Group: 2024 and 2025



¹ Estimates for 2024 may not match those published last year due to the implementation of Vintage 2025 population controls.

Note: There are no statistically significant changes between 2024 and 2025 presented in this figure.

Population as of March of the following year. Margins of error and other related estimates are available in Table 2 at

<www.census.gov/library/publications/2026/demo/p60-291.html>. Information on confidentiality protection, sampling error,

nonsampling error, and definitions is available at <<https://www2.census.gov/programs-surveys/cps/techdocs/cpsmar26.pdf>>.

Source: U.S. Census Bureau, Current Population Survey, 2025 and 2026 Annual Social and Economic Supplements (CPS ASEC).

(Data Management System [DMS] number: P-7534374 and Disclosure Review Board [DRB] approval number: CBDRB-FY26-0281).

Children Under the Age of 19

In 2025, 5.8 percent of children under the age of 19 did not have health insurance, which was not statistically different from 2024. Health insurance coverage rates for children under the age of 19 in

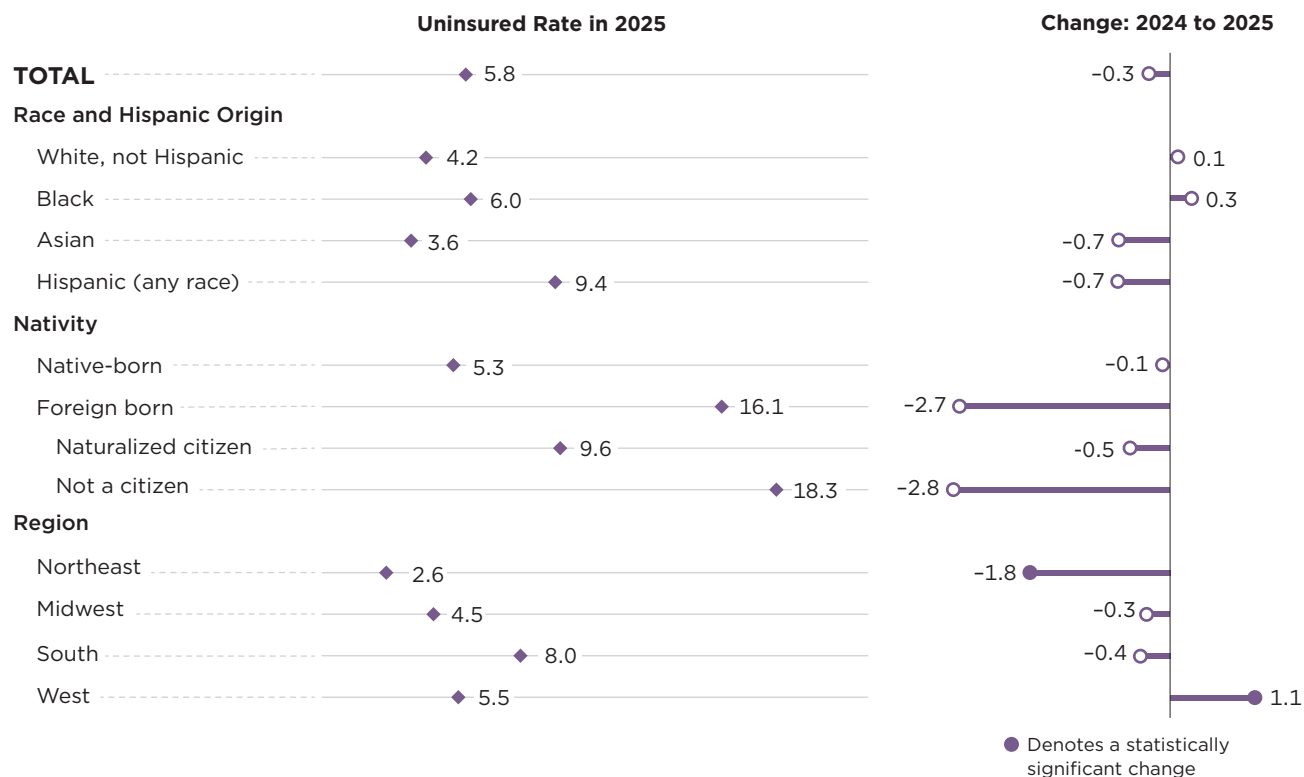
2025 differed by several factors, including race and Hispanic origin, nativity, and region (Figure 5).¹⁸

The uninsured rate in 2025 for children under the age of 19 in the Northeast (2.6 percent) decreased by 1.8 percentage points from

2024.¹⁹ This was the lowest rate among all regions. For children under the age of 19 in the West, the uninsured rate increased by 1.1 percentage points to 5.5 percent in 2025.²⁰

Figure 5.

Percentage of Children Under the Age of 19 Without Health Insurance Coverage by Selected Characteristics: 2024 and 2025



Note: Population as of March of the following year. Margins of error and other related estimates are available in Table 5 at www.census.gov/library/publications/2026/demo/p60-291.html. Information on confidentiality protection, sampling error, nonsampling error, and definitions is available at <https://www2.census.gov/programs-surveys/cps/techdocs/cpsmar26.pdf>.

Source: U.S. Census Bureau, Current Population Survey, 2025 and 2026 Annual Social and Economic Supplements (CPS ASEC). (Data Management System [DMS] number: P-7534374 and Disclosure Review Board [DRB] approval number: CBDRB-FY26-0281).

Working-Age Adults 19 to 64 Years Old

Working-age adults ages 19 to 64 may have different health insurance coverage from other age groups, in part because they only qualify for public coverage under limited circumstances. In 2025, 11.0 percent of adults ages 19 to 64 did not have health insurance coverage, which was not statistically different from the rate in 2024 (Figure 6).

Between 2024 and 2025, the uninsured rate for native-born working-age adults increased by 0.6 percentage points to 8.9 percent, while the uninsured rate for foreign-born working-age

adults decreased by 1.2 percentage points to 19.4 percent. Among foreign-born working-age adults, the uninsured rate for naturalized citizens (9.4 percent) increased, and the rate for noncitizens (28.9 percent) decreased.²¹

For adults ages 19 to 64 living in the Northeast, the uninsured rate decreased by 1.0 percentage point between 2024 and 2025.

ESTIMATES OF HEALTH INSURANCE COVERAGE: 2013 TO 2025

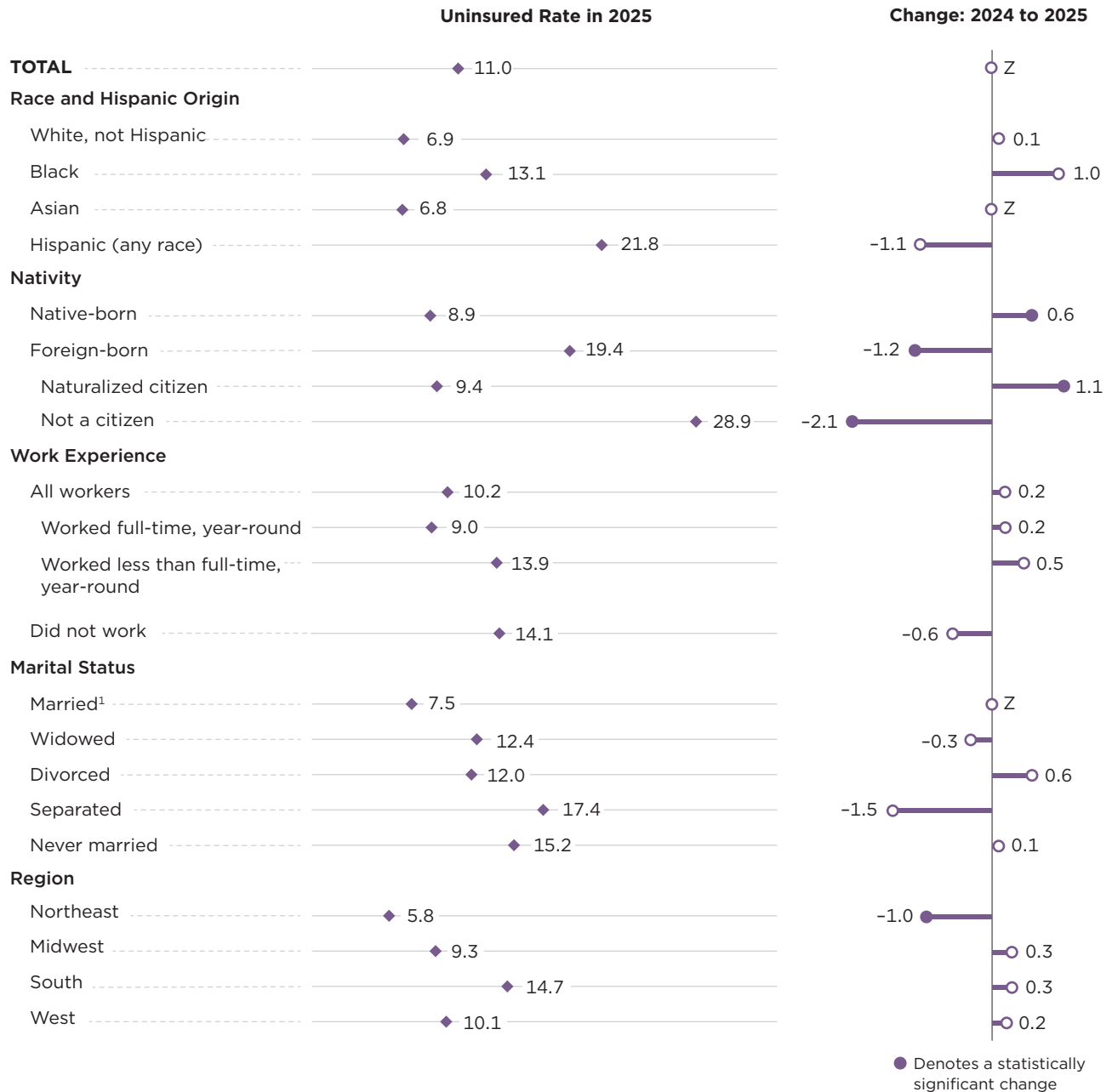
Overall Coverage Rate

Economic trends, demographic shifts, and changes in federal and

state policy provide important context for coverage levels and changes within social and demographic groups. Overall, the uninsured rate declined from 2013–2016 and 2020–2022; increased from 2017 to 2018; and did not significantly change year-to-year across the other years (Figure 7).²² In 2025, the uninsured rate continued to remain near historic lows at 7.9 percent and was not significantly different from the rate in 2024.²³

Figure 6.

Percentage of Adults Ages 19 to 64 Without Health Insurance Coverage by Selected Characteristics: 2024 and 2025



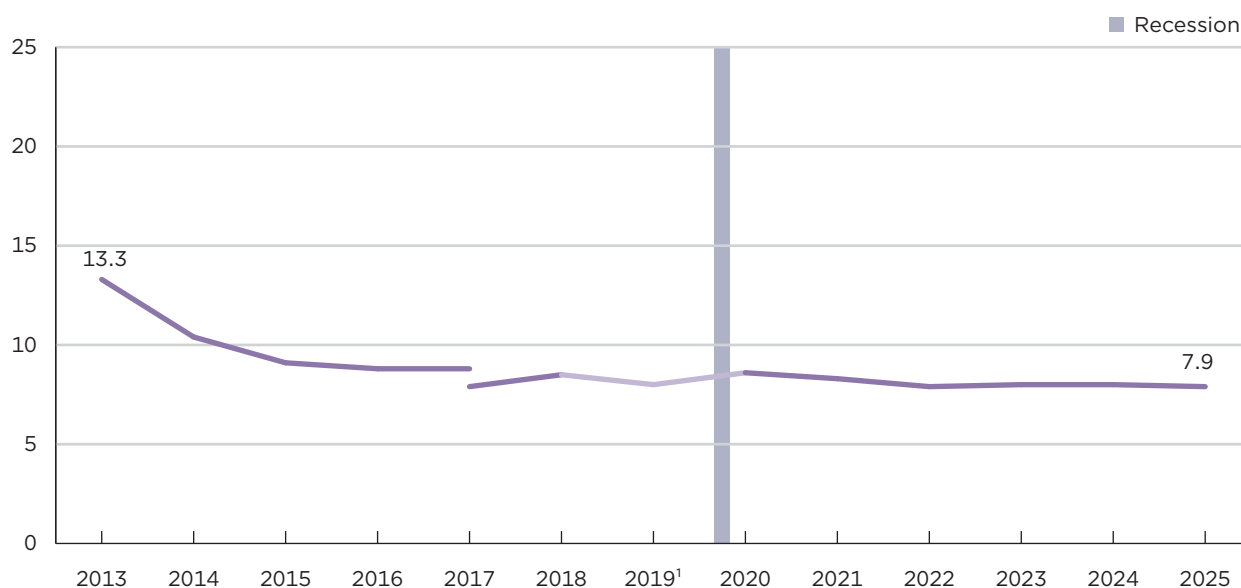
Z Rounds to zero.

¹ The combined category "married" includes three individual categories: "married, civilian spouse present," "married, U.S. armed forces spouse present," and "married, spouse absent."

Note: Population as of March of the following year. Margins of error and other related estimates are available in Table 5 at www.census.gov/library/publications/2026/demo/p60-291.html. Information on confidentiality protection, sampling error, nonsampling error, and definitions is available at <https://www2.census.gov/programs-surveys/cps/techdocs/cpsmar26.pdf>.

Source: U.S. Census Bureau, Current Population Survey, 2025 and 2026 Annual Social and Economic Supplements (CPS ASEC). (Data Management System [DMS] number: P-7534374 and Disclosure Review Board [DRB] approval number: CBDRB-FY26-0281).

Figure 7.
Percentage of People Without Health Insurance Coverage: 2013 to 2025



¹ The U.S. Census Bureau recommends using caution when making health insurance comparisons between calendar year coverage in 2019 (collected in 2020) and other years: <www.census.gov/content/dam/Census/library/working-papers/2023/demo/sehsd-wp-2023-27.pdf>.

Note: Population as of March of the following year. The data for 2017 and beyond reflect the implementation of an updated processing system. The data points are placed at the midpoints of the respective years. Information on recessions is available in the report appendix. Margins of error and other related estimates are available in Table 6 at <www.census.gov/library/publications/2026/demo/p60-291.html>. Information on confidentiality protection, sampling error, nonsampling error, and definitions is available at <<https://www2.census.gov/programs-surveys/cps/techdocs/cpsmar26.pdf>>.

Source: U.S. Census Bureau, Current Population Survey, 2018 Annual Social and Economic Supplement Bridge File and 2014 to 2026 Annual Social and Economic Supplements (CPS ASEC). (Data Management System [DMS] number: P-7534374 and Disclosure Review Board [DRB] approval number: CBDRB-FY26-0281).

SUMMARY

In 2025, the overall uninsured, private coverage, and public coverage rates did not significantly change from 2024.

There were changes in some types of insurance for the total population. For example, the overall Medicaid coverage rate

decreased by 0.5 percentage points, offsetting the 0.6 percentage-point Medicare coverage increase. Overall, private coverage continued to be more prevalent than public coverage.

People in some social and demographic groups experienced changes in their coverage rates

between 2024 and 2025. The public coverage rate for adults ages 19 to 64 decreased by 0.5 percentage points, driven by a decrease in Medicaid coverage for this group.²⁴ Additionally, private coverage dropped for adults age 65 and older, driven by declines in employment-based coverage and direct-purchase coverage.

ENDNOTES

¹ Additional details are available at <www.census.gov/library/working-papers/2026/demo/sehsd-wp2026-16.html>.

² Melissa Kollar and Zach Scherer, “Income in the United States: 2025,” Current Population Reports, P60-289, U.S. Census Bureau, Washington, DC, September 2026, <www.census.gov/library/publications/2026/demo/p60-289.html>, and Christina Bijou and Emily A. Shrider, “Poverty in the United States: 2025,” Current Population Reports, P60-290, U.S. Census Bureau, Washington, DC, September 2026, <www.census.gov/library/publications/2026/demo/p60-290.html>.

³ The margin of error for the uninsured rate in 2025 was 0.2.

⁴ Estimates of health insurance coverage status and coverage type for all people and by age and sex for 2024 and 2025 are available in Table 2 at <www.census.gov/library/publications/2026/demo/p60-291.html>.

⁵ Some people have more than one coverage type during the calendar year.

⁶ Between 2024 and 2025, the estimated percentage-point change of adults ages 19 to 64 with public coverage was not statistically different from the estimated percentage-point change of adults ages 19 to 64 with Medicaid coverage.

⁷ Between 2024 and 2025, the estimated percentage-point change of adults age 65 and older with employment-based coverage was not statistically different from the estimated percentage-point change of adults age 65 and older with direct-purchase coverage.

⁸ Infants born after the calendar-year reference period are excluded from estimates in this report.

⁹ The CPS ASEC also includes a measure of health insurance coverage held at the time of the interview. Although this measure of coverage cannot predict coverage in a given calendar year, it offers a snapshot of health insurance coverage early in the year when CPS ASEC data are collected. Additional statistics for this health coverage can be found in Table H-02 at <www.census.gov/data/tables/time-series/demo/income-poverty/cps-hi/hi.html>.

¹⁰ The final category includes CHAMPVA (Civilian Health and Medical Program of the Department of Veterans Affairs) coverage and care provided by the Department of Veterans Affairs (VA) and the military.

¹¹ The estimated proportion of the population age 65 and older with Medicare coverage did not statistically change between 2024 and 2025. The estimated percentage of the U.S. population age 65 and older, however, increased between 2024 and 2025.

¹² CHIP is a public assistance program that provides health insurance to children in families with income too high to qualify for Medicaid, but who may not be able to afford private health insurance.

¹³ Information on the Patient Protection and Affordable Care Act (ACA) is available online at <www.hhs.gov/healthcare/about-the-aca/index.html> and <www.govinfo.gov/content/pkg/PLAW-111publ148/pdf/PLAW-111publ148.pdf>.

¹⁴ Although most people age 65 and older held coverage through Medicare, 44.7 percent of people age 65 and older reported holding more than one type of coverage concurrently for some or all of calendar year 2025.

¹⁵ Federal surveys give respondents the option of reporting more than one race. Therefore, two basic ways of defining a race group are possible. A group, such as Asian, may be defined as those who reported Asian and no other race (the race-alone or single-race concept) or as those who reported Asian regardless of whether they also reported another race (the race-alone-or-in-combination concept). The body of this report (text and figures) shows data using the first approach (race alone). The online tables show data using both approaches. Primary use of the single-race population does not imply that it is the preferred method of presenting or analyzing data. The Census Bureau uses a variety of approaches. In this report, the terms “White, not Hispanic” and “non-Hispanic White” are used interchangeably and refer to people who are not Hispanic and who reported White and no other race. This report uses non-Hispanic White as the comparison group for other race and Hispanic origin groups. Since Hispanic individuals may be any race, data in this report for the Hispanic population overlap with data for race groups. Data users should be aware that the different race and Hispanic origin populations consist of many distinct groups that differ in socioeconomic characteristics, culture, and nativity, which may affect the interpretation of aggregate results. Data identifying Hispanic individuals were first collected in 1972 and for Asian and Pacific Islander individuals in 1987. More information is available at <www.census.gov/programs-surveys/cps.html>. Asian households are a relatively small proportion of the CPS ASEC sample. Further, the CPS ASEC does not use separate population controls for weighting the Asian sample to national totals. Together, these factors contribute to the large variances surrounding estimates for this group. As a result, the CPS ASEC may be unable to detect statistically significant differences between some estimates for the Asian population.

¹⁶ Private and public health insurance coverage rates by select characteristics for 2024 and 2025 are available in Table 3 at <www.census.gov/library/publications/2026/demo/p60-291.html>.

¹⁷ Private and public health insurance coverage rates by select characteristics for 2024 and 2025 are available in Table 4 at <www.census.gov/library/publications/2026/demo/p60-291.html>.

¹⁸ Information on how the U.S. Census Bureau defines each region is available at <www.census.gov/glossary/?term=Region>.

¹⁹ Uninsured rates for children under the age of 19 and adults ages 19 to 64 by select characteristics for 2024 and 2025 are available in Table 5 at <www.census.gov/library/publications/2026/demo/p60-291.html>.

²⁰ In 2025, the uninsured rate for all children under the age of 19 was not statistically different from the uninsured rate for children under the age of 19 living in the West.

²¹ In 2025, the uninsured rate for native-born adults ages 19 to 64 was not statistically different from the uninsured rate for naturalized citizens ages 19 to 64.

²² The series presented begins with data from 2013 after the implementation of the redesigned 2014 CPS ASEC questionnaire. For more information on the 2014 CPS ASEC redesign and for CPS ASEC health insurance data prior to 2013, refer to <www.census.gov/topics/health/health-insurance/guidance/cpsasec-redesign.html> and <www.census.gov/topics/health/health-insurance/data.html>. Figure 7 includes a series break after 2017, when an updated processing system was introduced for the CPS ASEC. Comparisons of health insurance coverage between calendar year coverage in 2019 (collected in 2020) and other years are not included. The U.S. Census Bureau recommends using caution when making health insurance coverage comparisons between 2019 and other years. The uninsured rates between 2018 and 2020 were not statistically different. Comparisons between 2018 and 2020 estimates use 2010 Census-based population controls. More information is available at <www.census.gov/content/dam/Census/library/working-papers/2023/demo/sehsd-wp-2023-27.pdf>.

²³ Uninsured rates for all people and by select characteristics for 2013 to 2025 are available in Table 6 at <www.census.gov/library/publications/2026/demo/p60-291.html>.

²⁴ Between 2024 and 2025, the estimated percentage-point change of all people with Medicaid coverage was not statistically different from the estimated percentage-point change of adults ages 19 to 64 with public coverage.

Appendix. Additional Information

SOURCE AND ACCURACY OF THE ESTIMATES

The Current Population Survey (CPS) is the longest-running survey conducted by the U.S. Census Bureau. The CPS is a household survey primarily used to collect employment data. The sample universe for the basic CPS consists of the resident civilian noninstitutionalized population of the United States. People in institutions, such as prisons, long-term care hospitals, and nursing homes are not eligible to be interviewed in the CPS. Students living in dormitories are included in the estimates only if information about them is reported in an interview at their parents' home. Since the CPS is a household survey, people who are homeless and not living in shelters are not included in the sample.

The CPS Annual Social and Economic Supplement (CPS ASEC), the source for the estimates in this report, collects data in February, March, and April each year, asking detailed questions categorizing income into over 50 sources. The key purpose of the survey is to provide timely and comprehensive estimates of income, poverty, and health insurance, and to measure change in these national-level estimates. The survey is the official source of national poverty estimates calculated in accordance with the Office of Management and Budget's Statistical Policy Directive 14.

The CPS ASEC collects data in the 50 states and the District of Columbia; these data do not

represent residents of Puerto Rico or the U.S. Island Areas.¹ The 2026 CPS ASEC sample consists of about 88,000 addresses. The CPS ASEC includes military personnel who live in a household with at least one civilian adult, regardless of whether they live on- or off-post. All other U.S. armed forces personnel are excluded. The estimates in this report are controlled to March 2026 independent national population estimates by age, sex, race, and Hispanic origin. Beginning with the data for 2020, population estimates are based on 2020 Census population counts and are updated annually after accounting for births, deaths, emigration, and immigration.

All 2024 estimates in this report and accompanying tables have been updated to incorporate Vintage 2025 population estimates and may not match previously published estimates. More information on the effect on 2024 estimates is available at <www.census.gov/library/working-papers/2026/demo/sehsd-wp2026-16.html> and details on the Vintage 2025 population estimate methodology are available at <<https://www2.census.gov/programs-surveys/popest/technical-documentation/methodology/2020-2025/methods-statement-v2025.pdf>>.

The estimates in this report (which may be shown in the text and figures) and in the accompanying online tables available at <www.census.gov/library/publications/2026/demo/p60-291.html> are based on responses

from a sample of the population and may differ from actual values because of sampling variability or other factors. As a result, apparent differences between the estimates for two or more groups may not be statistically significant. All comparative statements have undergone statistical testing and are statistically significant at the 90 percent confidence level unless otherwise noted.

In this report and the accompanying online tables, the variances of estimates were calculated using replication methods. For estimates prior to 2010, or as noted in historical tables, the Generalized Variance Function (GVF) method was used. More information on replicate weights, standard errors, income top-coding and data swapping on the public-use file, and changes to the CPS ASEC data file from the prior year is available at <<https://www2.census.gov/programs-surveys/cps/techdocs/cpsmar26.pdf>>.

Nonresponse Bias in the CPS ASEC

The Census Bureau administers the CPS ASEC each year between February and April by telephone and in-person interviews, with most data collected in March. Although the 2026 CPS ASEC was collected using standard procedures, response rates are still lower than they were before the pandemic. The weighted response rate for the 2026 CPS ASEC was 61.3 percent, compared to 62.0 percent for the previous year.

Since response rates remain below prepandemic levels, examining how respondents differ from nonrespondents is important, as this difference could affect the accuracy of the estimates. For more details on how sample differences and the associated nonresponse bias affect income and official poverty estimates, refer to the Research Matters blog, “Using Administrative Data to Evaluate Nonresponse Bias in the 2026 Current Population Survey Annual Social and Economic Supplement,” available at <www.census.gov/newsroom/blogs/research-matters/2026/09/evaluating-nonresponse-bias.html>.

CPS ASEC MODERNIZATION

The Census Bureau has begun a multiyear effort to modernize many of its surveys, including the CPS. Part of this involves adding an Internet Self-Response (ISR) mode to the CPS and then the CPS ASEC.

This project requires extensive review and testing to ensure that ISR is a viable collection mode for the CPS ASEC and that changes do not negatively affect the reliability and comparability of the estimates. The project schedule seeks to align the CPS ASEC modernization effort with that of the CPS to maintain continuity. However, the schedule and activities may change to accommodate funding availability, discovery of issues during testing and analysis, and project reprioritization.

More information about the ASEC modernization project and timeline is available on the Census Bureau’s CPS ASEC Modernization

Business Cycles—Recessions			
Peak month	Year	Trough month	Year
November	1948	October	1949
July	1953	May	1954
August	1957	April	1958
April	1960	February	1961
December	1969	November	1970
November	1973	March	1975
January	1980	July	1980
July	1981	November	1982
July	1990	March	1991
March	2001	November	2001
December	2007	June	2009
February	2020	April	2020

Source: National Bureau of Economic Research, <www.nber.org/research/data/us-business-cycle-expansions-and-contractions>.

Efforts webpage at <www.census.gov/programs-surveys/cps/about/modernization/asecmodernization.html> or by email at <demo.asec.modernization@census.gov>.

BUSINESS CYCLES—RECESSIONS

Business cycle peaks and troughs used to delineate the beginning and end of recessions, as shown in the text box “Business Cycles—Recessions,” are determined by the National Bureau of Economic Research (NBER), a private research organization. The data points in the time-series figures in this report use July as a reference. According to the NBER chronology, the most recent peak occurred in February 2020. The most recent trough occurred in April 2020. More information on business cycle dating is available at <www.nber.org/research/business-cycle-dating>.

ACCESSING HEALTH INSURANCE DATA

Additional CPS ASEC Estimates

Additional estimates from the CPS ASEC are available on the Census Bureau’s health insurance websites. The websites include detailed and historical tables, press releases, briefings, and working papers. The websites may be accessed through the Census Bureau’s homepage at <www.census.gov> or directly at <www.census.gov/topics/health/health-insurance.html>.

Public-Use Microdata

Public-use CPS ASEC microdata are available for data users of all skill levels.

Data users can create custom statistics from public-use microdata files using the Microdata Access Tool (MDAT) available at <<https://data.census.gov/mdat>>.

Microdata for the current CPS ASEC and earlier years are available online at <www.census.gov/data/datasets/time-series/demo/cps/cps-asec.html>. Technical methods have been applied to CPS microdata to avoid disclosing respondents' identities.

OTHER SOURCES OF HEALTH INSURANCE DATA

The Census Bureau recommends that people use the CPS ASEC for timely and thorough estimates of national health insurance coverage. However, the Census Bureau produces other data that are appropriate for subnational areas and longitudinal analysis. The American Community Survey (ACS) and the Small Area Health Insurance Estimates (SAHIE) program can be used for subnational health insurance estimates, while the Survey of Income and Program Participation (SIPP) provides monthly and longitudinal estimates.

American Community Survey

The ACS is an ongoing survey that collects comprehensive information on social, economic, and housing topics. Due to its large sample size, the ACS provides estimates at many levels of geography and for smaller population groups.

The Census Bureau presents annual estimates of health insurance coverage by state and other smaller geographic areas based on data collected in the ACS. Single-year estimates from the ACS are available for geographic areas with populations of 65,000 or more. Estimates of

health insurance coverage for all geographic areas, including census tracts and block groups, are available by pooling 5 years of ACS data. Health insurance estimates from the ACS are available at <<https://data.census.gov/>>.

Small Area Health Insurance Estimates

The SAHIE program uses statistical models to produce estimates of health insurance for states and all counties.

SAHIE methodology combines data from a variety of sources including administrative records, population estimates, and current surveys to provide consistent and reliable single-year estimates. In general, SAHIE estimates have lower variances than ACS estimates, but are released later because they incorporate these additional data into their models. Estimates from this program are available at <www.census.gov/programs-surveys/sahie.html>.

Survey of Income and Program Participation

The SIPP provides both monthly and longitudinal data about health insurance coverage, including sources of coverage, changes in coverage status, and plans at the individual, family, and household levels by following the same respondents over time. Whereas the CPS ASEC provides reliable estimates of the net change from 1 year to the next in the overall distribution of health insurance coverage for the whole population, it cannot show how these characteristics change

for the same person, family, or household. By collecting monthly data for the same respondents over multiple years, the SIPP makes it possible to observe how health insurance coverage changes at the individual level. This yields insights into the dynamic nature of these coverage transitions, as well as changes in health insurance status of U.S. residents. Estimates from these data are available at <www.census.gov/programs-surveys/sipp/library.html> and <www.census.gov/programs-surveys/sipp/data.html>.

QUESTIONS AND COMMENTS

For questions and assistance with health insurance data, contact the U.S. Census Bureau Customer Service Center at 1-800-923-8282 (toll-free), or search your topic of interest using the Census Bureau's "Question and Answer Center" at <www.census.gov/help>.

The Census Bureau also welcomes the comments and advice of data and report users. If you have suggestions or comments on this report, please email Liana Fox at <liana.e.fox@census.gov> or write to:

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ENDNOTE

¹ U.S. Island Areas include American Samoa, Guam, the Commonwealth of the Northern Mariana Islands, and the U.S. Virgin Islands.

